COUNTRY PROFILE AND MARKET INTELLIGENCE

Indonesia

Country profile



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Country overview and government

Indicator	Reported value
Capital	Jakarta[1]
Official language	Indonesian[1]
Population	About 285.7 million (2025 estimate)[2]
Area	1,904,569 sq km[1]
GDP (PPP), total	About 5.05 trillion current international dollars (2025)[3]
GDP (PPP), per capita	About 17,660 current international dollars (2025)[4]
Government type	Republic with a directly elected president[1,5]
Head of state and government	President Prabowo Subianto (sworn in on 20 October 2024)[1,5]
Vice president	Gibran Rakabuming Raka (sworn in on 20 October 2024)[5]
Currency	Rupiah (IDR)[6,7]
Urban population	59.4% of total (2025)[8]
Total fertility rate	2.12 births per woman (2024)[9]
Population aged 65 and over	7.5% (2025)[10]
Life expectancy at birth	71.3 years (2024)[11]

Indonesia had about 285.7 million people in 2025, spread across more than 6,000 inhabited islands, and 59.4 percent of them lived in urban areas.[2,8,12] The total fertility rate was 2.12 births per woman in 2024, and life expectancy at birth was 71.3 years.[9,11] Children aged 0 to 14 made up 24.2 percent of the population in 2025, while people aged 65 and over made up 7.5 percent, up from 5.9 percent in 2010.[10,13]

The age profile is therefore still young but aging steadily. That points to growing demand for cardiovascular, cancer and stroke care over the next decade, alongside continuing maternal, child and infectious disease needs. For a foreign academic medical center, the large urban centers are the most natural starting points for high-acuity collaboration, while remote and island regions are more likely to suit training and capacity-building work.

Government

Indonesia is a republic led by a directly elected president. Prabowo Subianto, a former military general, won more than 58 percent of the vote in the February 2024 election.[1] He was sworn in on 20 October 2024, together with Vice President Gibran Rakabuming Raka, for the 2024 to 2029 term.[5] Gibran is the eldest son of the previous president, Joko Widodo.[14] In his inauguration speech, Prabowo vowed to eradicate corruption and poverty and affirmed Indonesia's policy of non-alignment.[1]

Health Minister Budi Gunadi Sadikin represented the government when parliament passed the omnibus Health Law in July 2023 and still led the ministry in August 2026.[15,16] Danantara, a state-owned strategic entity, now leads operations of the Sanur health special economic zone, so counterparts for large hospital projects sit in the Health Ministry, state enterprises and Danantara as well as in private business groups.[17]

Economy and healthcare outlook

Economy

Indonesia's economy grew 5.11 percent in 2025, after 5.03 percent in 2024, and its GDP was about 5.05 trillion current international dollars at purchasing power parity.[3,18] The IMF's 2025 Article IV consultation, concluded by its Executive Board on 14 January 2026, described the economy as resilient, expected growth of 5.1 percent in 2026, and reported unemployment of 4.9 percent in 2025 and a poverty rate of 9.0 percent in 2024. It listed base metals, coal and palm oil among the main exports in 2024.[19] The World Bank's June 2026 forecast projects growth of 5.0 percent in 2026 and 5.2 percent in 2027 and 2028. It stresses productivity reforms, stronger private investment and better jobs.[20] Danantara is described as a state-owned strategic entity.[17]

The government aims to grow at eight percent a year by 2029 and to reach high-income status by 2045, under a 20-year plan for 2025 to 2045 delivered through five-year plans. The current plan focuses on human capital, digitalization and global competitiveness.[21] The gap between the eight percent goal and forecasts near five percent is wide.

Healthcare outlook

Policy direction is clear: prevention, more specialists, and stronger primary and referral care. The 2026 draft budget priorities include free health checks, faster tuberculosis control, more and better primary care facilities, revitalized regional hospitals, scholarships for medical education and stunting reduction.[44] World Bank financing is deploying 419 catheterization laboratories and is expected to upgrade 560 hospitals by 2029, and the Bank aims to raise Indonesia's universal health coverage service coverage index from 55 in 2025 to 62 by 2030.[12] The 2027 draft budget is nearly flat at Rp 244.9 trillion, up 1.7 percent, while the president plans to renovate 10,000 puskesmas and build 514 public health laboratories by 2030.[35]

Two constraints stand out. JKN is running deficits that BPJS Kesehatan says could lead to missed claim payments from July 2027 without government action,[31,45] and the specialist shortage will take years to close.[49] Demand for advanced services will therefore be shaped by what the insurer can pay, by how quickly specialists are trained, and by how fast foreign doctors can be licensed. For potential partners, the clearest opportunities lie in training linked to a defined service, measurable improvements in clinical quality, and implementation support for existing institutions. Each requires a local counterpart with both operating capacity and a credible financing plan.

Disease prevalence

Indicator	Reported value
Share of deaths from noncommunicable diseases	About two-thirds[12]
Probability of dying aged 30 to 70 from cardiovascular disease, cancer, diabetes or chronic respiratory disease	21.9% (2021)[22]
Stroke, share of deaths	19.42% (Ministry of Health)[23]
Ischemic heart disease, share of deaths	14.38% (Ministry of Health)[23]

Indicator	Reported value
Hypertension, adults aged 18 and over (measured)	30.8% (Indonesian Health Survey 2023); 34.1% in 2018[24]
Diabetes, people aged 15 and over (blood glucose measured)	11.7% (Indonesian Health Survey 2023); 10.9% in 2018[24]
Tuberculosis incidence	382 per 100,000 people (2024); 388 in 2023[25]
Stunting, children under five	22% (2023, World Bank series)[26]
Under-five mortality	17.7 per 1,000 live births (2024)[27]

Noncommunicable diseases such as diabetes, heart disease and cancer cause about two-thirds of deaths in Indonesia.[12] The Health Ministry has reported that stroke accounts for 19.42 percent of deaths and ischemic heart disease for 14.38 percent.[23] In May 2025 the health minister named stroke, heart disease, cancer and kidney disease as the four biggest killers, with about 300,000 stroke deaths a year.[28] The 2023 Indonesian Health Survey measured hypertension in 30.8 percent of adults aged 18 and over, down from 34.1 percent in 2018, and diabetes in 11.7 percent of people aged 15 and over, up from 10.9 percent.[24]

Infectious disease remains a second burden. Indonesia has the second-largest number of tuberculosis cases in the world, about 10 percent of the global total, and its incidence was 382 per 100,000 people in 2024.[25,29] Stunting affected 22 percent of children under five in 2023, and under-five mortality was 17.7 per 1,000 live births in 2024.[26,27] The mix of high chronic disease prevalence and unfinished infectious disease and nutrition work shapes demand for both advanced and basic services.

Healthcare delivery

Public healthcare system

Indonesia's public system rests on national health insurance (JKN), run by the state-owned insurer BPJS Kesehatan, which covered 282.7 million people, more than 98 percent of the population, at the end of 2025, although only 81.45 percent of members were active.[30,31] The World Bank reports that health service visits under the scheme grew from 378 million in 2020 to 686 million in 2024, and that out-of-pocket spending fell from 45 to 31 percent of total health spending.[12]

Care is delivered through community health centers (puskesmas), with more than 10,000 participating in free health checks in 2026,[16] and through hospitals run by the central government, local governments and private operators.[32] The Health Ministry has activated 300,000 community health posts (posyandu) and in 2023 began building vertical hospitals for cancer, heart, stroke and kidney care in Surabaya, Makassar, the new national capital in East Kalimantan and Jayapura.[33]

The free health check program began on 10 February 2025 at about 10,200 puskesmas.[34] It screened about 70 million people in 2025 and another 73.8 million in 2026 through 31 July, against a 2026 target of 130 million.[16] From 2027 the president plans to renovate 10,000 puskesmas in 7,285 sub-districts.[35]

Private healthcare system

Private operators ran 1,576 of Indonesia's 2,710 general hospitals, or 58 percent, in 2024.[32] The listed chains are growing. In 2025 Siloam reported revenue of Rp 12.84 trillion and net profit of Rp 1.11 trillion, Mitra Keluarga Rp 5.36 trillion and Rp 1.36 trillion, and Hermina Rp 7.13 trillion with net profit down 19.85 percent to Rp 429.55 billion.[36] Hermina plans three new hospitals in 2026 and more than 65 by 2030, and Mitra Keluarga plans two in Greater Jakarta and East Java.[36] By September 2025 the Astra group held 7.23 percent of Hermina and the Djarum group 3.6 percent.[37]

The premium segment is being targeted separately. Hermina's PIK 2 hospital focuses on non-JKN patients with premium facilities.[36] Mayapada Group, which Nikkei Asia describes as running one of Indonesia's largest private hospital chains, has committed Rp 6.91 trillion through 2032 to a Batam project with India's Apollo Hospitals.[7,38] State-owned Pertamedika IHC is the parent company of Bali International Hospital.[7] The World Bank Group's IFC also invests in private healthcare companies.[12]

Healthcare financing

Indicator	Reported value
Current health expenditure, share of GDP	2.70% (2023); 3.71% in 2021 during the pandemic[39]
Current health expenditure per capita	US$131.78 (2023, current US$)[40]
Government share of current health expenditure	53.6% (2023)[41]
Out-of-pocket share of current health expenditure	31.1% (2023); 50.9% in 2013[42]
Regional comparison, health expenditure share of GDP (2023)	Malaysia 3.96%, Thailand 4.54%, Vietnam 4.56%, Philippines 5.10%[43]
Health budget, 2026 draft state budget	Rp 244 trillion, 6.4% of state spending, up 15.8% on the 2025 outlook[44]
Health budget, 2027 draft state budget	Rp 244.9 trillion, up 1.7% on the 2026 outlook of Rp 240.8 trillion[35]
JKN fund deficit	Rp 17.13 trillion (2025); Rp 7.66 trillion in 2024[45]

Indonesia spends little on health relative to its neighbors. Current health expenditure was 2.70 percent of GDP in 2023, against 3.96 percent in Malaysia, 4.54 percent in Thailand, 4.56 percent in Vietnam and 5.10 percent in the Philippines.[43] The government financed 53.6 percent of spending in 2023, and households paid 31.1 percent out of pocket.[41,42] The 2026 draft state budget proposed Rp 244 trillion for health, 6.4 percent of state spending and 15.8 percent above the 2025 outlook, and the 2023 Health Law eliminated the mandatory health spending minimum.[15,44]

JKN's finances are the main strain. The JKN fund recorded a deficit of Rp 17.13 trillion in 2025, against Rp 7.66 trillion in 2024.[45] In June 2026 BPJS Kesehatan said monthly claims of Rp 16 to 16.5 trillion exceed premium income of about Rp 14 trillion, and that it could begin missing claim payments by July 2027 without government intervention.[31] These figures make reimbursement reliability a practical consideration for hospital expansion and cross-border partnerships, particularly where new services depend on JKN patients rather than private payment.

Workforce and education

Indicator	Reported value
Physicians per 1,000 people	0.52 (2023); 0.69 in 2021[46]
Nurses and midwives per 1,000 people	3.24 (2023)[47]
Hospital beds per 1,000 people	1.37 (2023)[48]
General practitioners	About 160,000, with about 120,000 more needed for the WHO ratio of 1 per 1,000 (Health Ministry, reported by Nikkei Asia, early 2025)[7]
Specialist shortage	About 29,000 (Health Minister, early 2025); about 70,000 projected through 2032[7,49]
Specialists produced per year	About 2,700, against about 32,000 needed (Health Ministry, February 2026)[50]

Indonesia has few doctors for its size. In 2021 it had 0.69 physicians per 1,000 people, against 2.27 in Malaysia, 1.11 in Vietnam, 0.79 in the Philippines and 2.78 in Singapore, and the separately reported 2023 figure was 0.52.[46] Nikkei Asia reported in early 2025 that Indonesia had about 160,000 general practitioners for 280 million people and that the ministry said it needed 120,000 more to meet the WHO recommendation of one doctor per 1,000 residents.[7] Nurses and midwives numbered 3.24 per 1,000 people and hospital beds 1.37 per 1,000 in 2023.[47,48]

The gap is sharpest among specialists. The minister cited a shortage of about 29,000 specialists, and the ministry says the country produces about 2,700 a year against an ideal requirement of 32,000.[7,50] It projects a shortage of around 70,000 specialists through 2032.[49]

Education

Output is the constraint: about 2,700 specialists graduate a year against about 32,000 needed.[50] President Prabowo has set a target of 70,000 more specialists, which the health and higher education ministers estimate would take 10 to 15 years, although the president wants it faster.[49] The main response is hospital-based specialist training, which prioritizes general practitioners from regions without specialists and which the health minister says will not charge local trainees.[50]

The 2023 Health Law also changed licensing. It moved the authority to issue medical competency certificates from medical associations to the government and allows foreign doctors to practice in the country.[15] In practice, the minister told Nikkei Asia that granting foreign doctors a license remained difficult, partly because the Indonesian Medical Association wants foreign recruits to speak Indonesian and recruitment to follow a clear mapping of needs. The government has also offered more scholarships for medical students.[7]

International training provides a complementary route. In January 2025 the ministry sent 27 doctors for one-year fellowships in China and Japan, covering interventional cardiology and neurology.[51] Such programs can strengthen specific clinical teams while hospital-based education expands domestic training capacity.

Competitive landscape

Foreign participation includes ownership, operations and clinical partnerships. Malaysia-based A1Health owns nine Indonesian hospitals, including RS Premier facilities.[52] Australia's Icon operates Bali International Hospital's cancer service; SingHealth is its academic partner.[53,54] Mayo Clinic consulted on BIH's planning, but current work is unconfirmed.[7] Apollo partners with Mayapada in Batam.[38] Other relationships concentrate on clinical training, research and advisory services.

Active partnerships in Indonesia





PARTNERSHIPS BY CITY

1	Medan	2
2	Bukittinggi	1
3	Greater Jakarta	13
4	Tegal / Brebes	1
5	Semarang	1
6	Surabaya	2
7	Denpasar	5

Active foreign health-system partnerships by city of the partner in Indonesia. Source: CSI partnerships tracker, verified 2026-10-07. Boundaries: Natural Earth; not an official depiction. Mapped: 22 active relationships with identified Indonesian sites. Five national or overseas-training programs are omitted. Greater Jakarta and Tegal/Brebes are grouped; one relationship may span cities. The planned Batam hospital is not shown.

Outbound care remains substantial. Former President Joko Widodo estimated that more than 1 million citizens travel abroad annually, costing Indonesia US$11.5 billion; Malaysia and Singapore are prominent destinations.[7] These are officials' estimates, which vary across reports.[17,38] Indonesia is expanding domestic alternatives through the Sanur health special economic zone, inaugurated in June 2025, and Batam's planned 250-bed Mayapada-Apollo hospital, with full operations targeted for late 2027.[17,38] These projects combine local investment with foreign expertise, while payer constraints and specialist availability will affect their reach.

Institution directory

Confirmed active partnerships by institution. Institutions 1 to 13 of 25; continued on the next page.

Institution and location	Nature of collaboration	Evidence
Cedars-Sinai United States	Training and education	2024-12[55,56,57]
Harvard Medical School and Brigham and Women's Hospital (Program in Global Primary Health Care) United States	Research collaboration	2026-05[58,59]
MD Anderson Cancer Center United States	Advisory and consulting, Government agreement	2026-03[60,61,62,63]
Mayo Clinic United States	Clinical affiliation	2024-12[64,65,66]
St. Jude Children's Research Hospital United States	Training and education	2026-04[67,68]
Apollo Hospitals India	Clinical affiliation	2026-01[69,70,71]
Asia OneHealthcare (A1Health; formerly Columbia Asia Healthcare) Malaysia	Ownership or management	2026-02[52,72]
Chung Shan Medical University (Taiwan) Taiwan	Training and education	2025-08[73]
Hong Kong Asia Medical Group (HKAMG) Hong Kong	Clinical affiliation	2025-04[54,74]
Icon Group (Icon Cancer Centre) Australia	Ownership or management	2026-03[53,75,76]
International University of Health and Welfare (IUHW) Japan	Training and education	2026-04[77,78]
Kobe University Japan	Research collaboration	2025-02[79]
Monash Health Australia	Training and education	2026-07[80,81]

Institution directory continued

Institutions 14 to 25 of 25. Evidence dates identify the latest dated source, not contract expiration.

Institution and location	Nature of collaboration	Evidence
Ramathibodi Hospital, Mahidol University Thailand	Training and education	2025-05[82]
Sapporo Cardiovascular Clinic Japan	Clinical affiliation	2025-04[51,54]

Institution and location	Nature of collaboration	Evidence
Seoul National University Hospital South Korea	Clinical affiliation, Training and education	2026-04[83,84,85]
SingHealth Singapore	Advisory and consulting	2025-04[54,86,87]
SingHealth Duke-NUS Global Health Institute (SingHealth and Duke-NUS Medical School) Singapore	Training and education	2025-07[88,89]
Sun Medical Center / Sun Healthcare International South Korea	Clinical affiliation	2025-06[90,91]
Taipei Veterans General Hospital Taiwan	Clinical affiliation	2025-05[92]
Tokushukai Medical Group Japan	Clinical affiliation	2026-04[93,94,95]
University of Glasgow and CRUK Scotland Institute United Kingdom	Training and education	2026-05[96]
Wuhan Asia Heart Hospital China	Training and education	2024-11[97]
Zhongda Hospital (Southeast University) China	Training and education	2025-01[51]
Zhongshan Hospital (Fudan University) China	Training and education	2025-01[51,97,98]

Only partnerships with dated evidence of activity in the last 24 months are listed. Unclear and recently ended partnerships are in the CSI partnerships tracker.

Recent developments

2026-08-04 Free health checks reach 73.8 million participants in 2026

The ministry reported 73.8 million free health checks in 2026 through 31 July, against a 130 million annual target, following about 70 million in 2025. It is strengthening follow-up for high blood pressure, high blood sugar and high cholesterol.[16]

2025-06-25 President Prabowo inaugurates Sanur health special economic zone in Bali

The Sanur zone, Indonesia's first health-focused special economic zone, centers on Bali International Hospital and is operated under Danantara.[17] Mayo Clinic previously consulted on hospital development; BIH's April 2025 partner announcement names Icon for cancer services and SingHealth for continuing medical education.[7,54]

2025-08-27 Mayapada and Apollo break ground on 250-bed international hospital in Batam

Mayapada Healthcare began building a Rp 1 trillion, 250-bed hospital in Batam's health special economic zone in partnership with Apollo Hospitals India, with full operations targeted for the end of 2027.[38]

2026-06-09 BPJS Kesehatan warns it could miss claim payments from July 2027

The insurer said it is running monthly deficits of about Rp 2 trillion and expects a Rp 20 trillion government capital injection.[31] The JKN fund's 2025 deficit was Rp 17.13 trillion, up 123.8 percent from 2024.[45]

2026-08-14 2027 draft budget holds health at Rp 244.9 trillion and funds puskesmas renovation

The draft health budget is up 1.7 percent on the 2026 outlook of Rp 240.8 trillion.[35] President Prabowo said 10,000 puskesmas in 7,285 sub-districts would be renovated from 2027 and 514 public health laboratories built by 2030.[35]

CSI assessment

Everything below comes from Cedars-Sinai's internal data (patient referral records, finance, relationship history) and human judgment, not from the research agents. Fields left blank are marked and must be completed by the analyst before this profile is used.

Patient Volume & Revenue (most recent FY)

[MANUAL INPUT REQUIRED]

Referral Sources

[MANUAL INPUT REQUIRED]

Relationship History

[MANUAL INPUT REQUIRED]

Complications

[MANUAL INPUT REQUIRED]

Key Contacts

[MANUAL INPUT REQUIRED]

Opportunities

[MANUAL INPUT REQUIRED]

CSI Recommendation

[MANUAL INPUT REQUIRED]